

15. Pelvic Organ Prolapse and POP-Q

Companion reading handout for simultaneous listening.

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Before the clinical content, place this paper in its correct frame. ACOG Practice Bulletin 214 is an interim update, and the update reflects the U.S. Food and Drug Administration order to stop the sale of transvaginal synthetic mesh products for pelvic organ prolapse repair. That matters because the paper still teaches mesh evidence, mesh complications, selective use, consent, and surgeon training. But the regulatory frame is not routine transvaginal mesh availability for primary POP repair. Keep that in mind whenever mesh comes up.

Why POP is common but not trivial

Pelvic organ prolapse, or POP, is common and benign, but say "benign" carefully. Benign does not mean harmless. It means nonmalignant. POP is not cancer and is not, by itself, a malignant disease process, but it can still produce vaginal bulge, pelvic pressure, voiding dysfunction, defecatory dysfunction, sexual dysfunction, and major quality-of-life burden.

That is the first mental model: POP is not simply "an organ falling." It is a failure of pelvic support that becomes clinically important when anatomy turns into symptoms or functional disruption.

ACOG gives the first hard burden number early: women in the United States have a 13 percent lifetime risk of undergoing surgery for POP. That means that across a woman's life, roughly 13 out of 100 women are expected to have an operation for prolapse. It does not mean 13 percent have symptomatic prolapse today, and it does not mean every anatomic prolapse needs surgery. Later, ACOG gives the contrast that makes this clear: symptom prevalence is about 3 to 6 percent, while examination prevalence is about 41 to 50 percent. So the 13 percent number is best understood as cumulative lifetime surgical burden, not current disease prevalence.

ACOG also says the peak incidence of POP symptoms is in women aged 70 to 79 years. Do not turn that age band into a magic biologic cutoff. It is an epidemiologic peak. The mechanism that makes it plausible is cumulative support failure: childbirth-related levator injury, enlargement of the levator hiatus, endopelvic connective-tissue failure, apical support loss, aging tissue, body mass index, and chronic increases in intra-abdominal pressure can accumulate for decades before a woman experiences a bothersome bulge, pressure, urinary symptom, bowel symptom, or sexual symptom.

And because the population is aging, ACOG anticipates that by 2050 the number of women experiencing POP will increase by about 50 percent. That is a population-burden statement, not an individual-risk multiplier. It means more older women in the population, more symptomatic POP, more pessary care, more counseling, more operations, and more need for clinicians who understand the anatomy and the tradeoffs.

Anatomy and symptom-driven disease

Pelvic organ prolapse is descent of one or more aspects of the vagina and uterus: the anterior vaginal wall, posterior vaginal wall, uterus or cervix, or the apex of the vagina. The apex depends on surgical history. If the uterus is present, the apical reference is the cervix and related fornices. After hysterectomy, the apical reference is the vaginal vault, especially the cuff scar.

Traditional terms like cystocele, rectocele, and enterocele are useful, but they are organ-based shorthand. A cystocele means descent of the bladder with the anterior vaginal wall. A rectocele means the rectum protrudes into the vaginal lumen through weakened posterior vaginal and rectovaginal support. An enterocele is different: it is a true hernia, a peritoneal sac, often with small bowel, descending toward the vaginal axis, commonly through the apical or rectovaginal space. But on exam, we may not know with certainty which organ is behind the prolapsed vaginal epithelium. What we can measure with certainty is the vaginal segment that descends. That is why POP-Q prefers anterior, apical, and posterior measured points over relying only on labels like cystocele and rectocele.

This leads to the second mental model: separate anatomic prolapse from clinically important prolapse. Mild descent is common and should not automatically be called pathologic. ACOG is explicit: POP should be considered a problem only if it causes prolapse symptoms, usually pressure with or without bulge, sexual dysfunction, lower urinary tract dysfunction, or bowel dysfunction.

The prevalence gap proves why that distinction matters. When POP is counted by symptoms, prevalence is about 3 to 6 percent. When it is counted by examination, prevalence is about 41 to 50 percent. Those are not interchangeable populations. The exam can detect descent that the woman does not feel, does not report, and does not experience as a problem. So whenever someone says "prevalence of POP," ask: prevalence by symptom report, or prevalence by pelvic exam?

The clinical consequence is simple: do not treat a POP-Q grid. Treat the patient. The grid documents anatomy. Symptoms, function, risk, bother, and goals determine whether that anatomy needs intervention.

POP-Q without the fog

POP-Q is a coordinate system. Its zero point is the hymen. Measurements above the hymen, inside the vagina, are negative. Measurements beyond the hymen, outside or distal to the hymenal plane, are positive. The exam is performed during maximal Valsalva, because the point is to document maximal descent.

There are six vaginal points and three additional measurements. On the anterior vaginal wall, Aa is 3 centimeters proximal to the external urethral meatus, and Ba is the most prolapsed portion of the anterior vaginal wall. At the apex, C is the leading edge of the cervix or the vaginal cuff. D is the posterior fornix in a woman who still has a cervix. D is omitted after hysterectomy, and it helps distinguish suspensory failure from cervical elongation. On the posterior vaginal wall, Ap is 3 centimeters proximal to the posterior hymen, and Bp is the most prolapsed portion of the posterior wall. Then come the three measurements: genital hiatus, perineal body, and total vaginal length.

If that sounds like too many letters, use the map: anterior wall, apex, posterior wall, and outlet. Aa and Ba are anterior. C and D are apical. Ap and Bp are posterior. Genital hiatus and perineal body describe the outlet. Total vaginal length tells you the available vaginal depth with the prolapse reduced.

The staging ladder is built from the most distal point relative to the hymen. Stage 0 means no prolapse by POP-Q criteria. Stage I means there is descent, but the leading point remains more than 1 centimeter above the hymen, less than minus 1. Stage II means the leading point is within 1 centimeter above or below the hymen, from minus 1 through zero to plus 1. Stage III means the leading point is more than 1 centimeter beyond the hymen, but not complete or near-complete eversion. Stage IV means complete or nearly complete eversion, defined by maximum descent at least total vaginal length minus 2 centimeters.

Notice what those numbers are. The hymen is the reference plane. The 1 centimeter boundaries are standardization boundaries. They are not biologic switches. They do not mean symptoms suddenly begin at exactly minus 1, zero, or plus 1.

ACOG gives a separate symptom-threshold number: most women feel symptoms when the leading edge reaches 0.5 centimeters distal to the hymenal ring. In POP-Q language, that is plus 0.5, half a centimeter beyond the hymenal plane. The leading edge is whichever compartment is most dependent on exam: anterior wall, posterior wall, cervix, or cuff. This 0.5 centimeter number is not a stage boundary; it lies inside Stage II. It is also not a threshold for every urinary, bowel, pain, or sexual symptom. The evidence is strongest for bulge or protrusion symptoms. In other words: when the leading edge passes about half a centimeter beyond the hymen, bulge awareness becomes much more likely, but the rest of the symptom story still needs its own clinical assessment.

Mechanism, natural history, and evaluation

POP usually reflects global pelvic floor dysfunction. Most women do not have one isolated compartment quietly failing on its own. The same support system is shared across compartments: the levator ani muscles, the endopelvic connective tissue, the cardinal-uterosacral complex, paravaginal attachments, and distal perineal support. If those systems fail, the result is often combined anterior, apical, and posterior descent, with one compartment simply leading clinically.

Risk factors make sense if you keep that support model in mind. Vaginal delivery and increasing parity matter because childbirth can injure the levator ani, stretch the hiatus, and damage connective tissue. Age matters because tissue reserve changes over decades. Obesity matters because it is chronic increased pressure on a weakened support system. Chronic constipation and straining matter because they repeatedly increase downward force. Advanced preoperative stage matters because Stage III or IV prolapse tells you the baseline support failure is already severe. These are not random risk factors. They are different ways of damaging support, loading support, or revealing severe failure.

Natural history is also important. ACOG notes that in one study following women with symptomatic untreated POP for an average of 16 months, 78 percent had no change in the leading edge. For women who do not want treatment, most will have no change or only a small increase over the next year. This does not mean prolapse never progresses. It means observation is a real option when symptoms are tolerable, function is acceptable, and there is no urgent complication.

Evaluation starts with the patient's symptoms and goals. Ask about bulge and pressure. Ask whether she sees or feels tissue at the introitus. Ask about voiding: stress incontinence, urgency, incomplete emptying, retention, splinting to void. Ask about bowel function: straining, incomplete emptying, obstructed defecation, fecal incontinence, splinting to defecate. Ask about sexual function: dyspareunia, avoidance, bother, and whether vaginal intercourse is desired in the future. Then examine with POP-Q or a simplified POP-Q so the anatomy is documented objectively before treatment.

When ACOG asks how to evaluate suspected POP, the first answer is not "measure everything." It is: take a thorough history, assess symptom severity, perform a physical examination, and clarify the patient's goals for treatment. That sequence is important. History tells you what the patient is actually experiencing. Symptom severity tells you whether the anatomy is clinically meaningful. Physical examination tells you which compartments are descending and how far. Goals for treatment tell you what problem the patient wants solved: reassurance, a pessary, preserved sexual function, uterine preservation, avoidance of surgery, or durable surgical repair. In POP, the goal is not to correct a number; the goal is to solve the patient's problem with the least inappropriate tradeoff.

Assess pelvic floor muscle tone because POP is not only a fascial map; it is also a muscle-and-nerve support problem. The levator ani and coccygeus help hold the

pelvic floor closed and provide lifting and closing forces. If that active support is weak or dysfunctional, the levator hiatus can enlarge and descent becomes easier. So tone assessment asks how much active support remains.

ACOG's default is conservative: before treatment, most women with POP do not need a battery of additional tests. But that statement rests on the word complete. The baseline evaluation must include gynecologic, urologic, and defecatory history, a physical examination, and objective prolapse documentation when treatment is being planned. If that workup explains the patient's symptoms and anatomy, extra tests usually add little. The test is ordered when a specific problem asks for it, not because POP automatically requires imaging, urodynamics, or laboratory screening.

Finally, if the story and the exam do not concur, escalate thoughtfully. First make sure the exam was dynamic enough: Valsalva, cough, and, when needed, standing examination. If the mismatch persists, more specific imaging or referral to urogynecology may be needed. Think of severe bulge symptoms with little supine descent, urinary or bowel dysfunction out of proportion to the observed prolapse, suspected alternate pathology, or complex recurrent prolapse. Referral is not failure. It is the correct move when the patient's symptoms, anatomy, or surgical needs exceed what the initial assessment can confidently explain.

The resident reflex is this: first define the anatomy, then define the bother, then define the goal. Do not reverse the order.

Conservative treatment begins with the pessary. A pessary is not a consolation prize after surgery is declined. It is a legitimate first-line alternative. ACOG states that women considering POP treatment should be offered a vaginal pessary as an alternative to surgery. About two thirds of symptomatic patients initially choose pessary management, and up to 77 percent continue use after 1 year. When fitted and followed by an experienced clinician, complication rates are low, though vaginal discharge, odor, bleeding, ulceration, and erosions can occur and require follow-up.

If erosion or irritation persists, ask what is still applying pressure. The answer may be maintenance interval, size, or type. ACOG recommends more frequent pessary changes or a different pessary. That makes sense because pessary fitting is partly trial and error: a device can support the prolapse but still create focal pressure, or a patient may need a shorter interval before tissue inspection and cleaning.

Pessary logic is especially important for a woman who desires future pregnancy. The point is reversibility. A pessary gives mechanical support without committing her to reconstructive surgery before pregnancy changes the pelvic floor again. Control symptoms now, preserve options later.

Surgery as anatomy plus tradeoff

The cleanest surgical model is anatomy plus tradeoff. Surgery should be mapped to the failed compartment, the patient's goals, the patient's comorbidities, and the acceptable risk. Some operations restore apical support. Some repair the anterior wall. Some repair the posterior wall and perineal body. Some use native tissue. Some use mesh or graft. Some preserve vaginal function. Some intentionally close or narrow the vagina.

Table 1 is best heard as a compartment map. For upper vaginal or apical prolapse, the main operations listed are abdominal sacral colpopexy, uterosacral ligament suspension, and sacrospinous fixation. Abdominal sacral colpopexy is most commonly used for recurrent cystocele, vault prolapse, or enterocele. Uterosacral suspension and sacrospinous fixation are apical support operations used at hysterectomy or for posthysterectomy vaginal vault prolapse. For anterior wall prolapse, the table lists anterior vaginal repair, or anterior colporrhaphy, classically for bladder or urethral descent into the vagina. For posterior wall prolapse, it lists posterior colporrhaphy and perineorrhaphy, for rectocele and deficient perineal body. And then the table lists vaginal repair with synthetic mesh or biologic graft augmentation for selected anterior, apical, or combined defects, while noting that this is not routinely recommended. So the table is not a menu to memorize; it is a map from compartment to operation.

The apical principle is one of the most important surgical ideas in the paper. The apex is not just another compartment. It is the upper support point for the vaginal axis. If the apex is unsupported, an anterior repair may look locally repaired while the upper vagina continues to descend. That is why ACOG says vaginal hysterectomy alone is not adequate treatment for uterine prolapse. If the uterus is prolapsing, removing the uterus does not automatically repair the Level I support failure. The upper vagina and cervix are normally supported by the cardinal-uterosacral complex. At hysterectomy for uterine prolapse, the apex still needs suspension, usually to the uterosacral or sacrospinous ligaments, to reduce recurrent POP.

Uterosacral suspension is an anatomic operation: take the vaginal apex and attach it to the uterosacral ligament support system. ACOG describes two ways to do it: attach the apex bilaterally to the ipsilateral uterosacral ligaments, or attach the apex to a uterosacral ligament complex plicated in the midline. The clinically important detail is that an adequate segment of uterosacral ligament must be secured to the vagina, often near the midportion of the ligament close to the ischial spine. This is not a symbolic stitch. It has to capture real load-bearing support.

The other native-tissue anchor is the sacrospinous ligament. This is still a vaginal apical suspension, but instead of using the uterosacral complex, the apex is fixed to the sacrospinous ligament, the tendinous part of the coccygeus complex near the ischial spine. Its appeal is that it is a transvaginal, usually extraperitoneal route and can give durable apical support. Its tradeoffs are different: exposure can be difficult, the vagina may be deflected laterally toward the fixation site, tension can be a

problem in a shortened vagina or repeat repair, and the dissection is near pudendal and sciatic neurovascular structures.

For native-tissue apical repair, ACOG gives a strong Level A anchor: uterosacral ligament suspension and sacrospinous ligament fixation are equally effective surgical treatments, with comparable anatomic, functional, and adverse outcomes. The trial numbers are almost identical at 2 years: surgical success 64.5 percent with uterosacral suspension versus 63.1 percent with sacrospinous fixation. Serious adverse events were also similar: 16.5 percent versus 16.7 percent. So the question is not "which one wins?" The point is that both are evidence-supported native-tissue apical repairs, and the choice depends on anatomy, route, surgeon skill, and patient-specific risk.

Anterior and posterior repair must be handled differently. For posterior vaginal wall prolapse, ACOG's Level A conclusion is sharp: synthetic mesh or biologic grafts in transvaginal posterior repair do not improve outcomes. In fact, the evidence can favor native tissue. In two randomized trials comparing native tissue with biologic graft for posterior repair, objective failure at 1 year was lower in the native-tissue group: 10 percent versus 21 percent. Another trial found worse anatomic outcomes with posterior biologic graft repair than traditional repair: 46 percent versus 14 percent. Add posterior-incision mesh exposure risk, and the teaching is clear: for primary posterior vaginal wall prolapse, routine augmentation does not buy the patient a better outcome.

For posterior vaginal wall prolapse, the route should match the pathology. The defect is in the posterior vaginal support plane, the rectovaginal fibromuscular tissue that lets the rectum bulge forward into the vagina. A transvaginal incision gives direct access to that vaginal support layer. A transanal repair works from the rectal side, usually by plicating rectal mucosa or rectal muscularis and reducing the rectal vault; it may help selected defecatory dysfunction, but it does not directly reconstruct the vaginal support plane as well. That is why ACOG favors the transvaginal route for posterior vaginal wall prolapse.

The systematic review gives the numbers. Compared with transanal repair, transvaginal posterior repair had fewer recurrent prolapse symptoms, with a relative risk of 0.4 and a 95 percent confidence interval from 0.2 to 1.0. It had lower clinical recurrence, with a relative risk of 0.2 and a confidence interval from 0.1 to 0.6. And on defecography, the mean rectocele depth was 1.2 centimeters smaller, with a confidence interval from 0.3 to 2.0 centimeters smaller. Translate that: the anatomic outcomes clearly favor the transvaginal route; the symptom recurrence result also points that way, though its confidence interval reaches 1.0, so it is the most borderline of the three.

Anterior polypropylene mesh is different, and it must be taught as a tradeoff. Compared with native-tissue anterior repair, polypropylene mesh augmentation can improve anatomic and some subjective outcomes. But it increases morbidity: longer

operating time, greater blood loss, and increased repeat surgery in a composite outcome that includes prolapse, stress urinary incontinence, and mesh exposure. ACOG's vaginal mesh exposure or erosion figure is about 12 percent, with 8 percent requiring repeat surgery for exposure within 3 years. The rate of dyspareunia after vaginal mesh prolapse surgery is about 9 percent. So the accurate teaching is not "mesh works" and not "mesh never." It is selective risk-benefit counseling.

Abdominal sacrocolpopexy is another tradeoff operation. It suspends the vaginal apex to the anterior longitudinal ligament of the sacrum using mesh or graft. It may be considered for shortened vaginal length, intra-abdominal pathology, or risk factors for recurrence such as age under 60, Stage III or IV prolapse, or body mass index greater than 26. Compared with vaginal native-tissue apical repair, mesh abdominal sacrocolpopexy has better anatomic success, with pooled odds ratio 2.04. But the same comparison shows more complications: ileus or small bowel obstruction 2.7 percent versus 0.2 percent, thromboembolic events 0.6 percent versus 0.1 percent, and mesh or suture complications 4.2 percent versus 0.04 percent. Long-term CARE trial follow-up estimated mesh complications at 10.5 percent. So sacrocolpopexy may reduce recurrence risk, but it buys that durability with abdominal and mesh-related risk.

Sacrocolpopexy is defined by the support construction, not by the skin incision or robot. The same basic operation, suspending the vaginal apex to the sacrum with graft or mesh, can be done open, laparoscopically, or laparoscopically with robotic assistance. It can also be combined with supracervical hysterectomy, total hysterectomy, or no hysterectomy, depending on whether the uterus is present and what uterine plan is chosen. So teach this as route and platform flexibility: the target is apical suspension; laparoscopy and robotics are ways to access and execute it.

The randomized trial strengthens the same message. In the RCT, open surgery had more blood loss by a mean difference of 184 milliliters, with a 95 percent confidence interval from 96 to 272. Laparoscopy reduced inpatient stay by a mean difference of 0.9 days, with a 95 percent confidence interval from 0.1 to 1.7. Because these intervals do not cross zero, the trial supports real differences favoring laparoscopy for blood loss and hospital stay. The RCT matters because it reduces selection bias; it still does not remove the need for laparoscopic expertise.

Robotics should be taught with discipline. A robot may shorten the learning curve and improve surgeon ergonomics; those are real platform advantages for the operator. But ACOG states that this has not translated into better short-term patient outcomes. So the resident should separate two questions: does the robot make the operation easier to perform, and does it make the patient do better? In the evidence ACOG cites, the first may be yes; the second has not been demonstrated.

Obliterative surgery is a different category. These procedures narrow, shorten, or completely close the vagina. They should be considered first-line surgical treatment for women with significant medical comorbidities who do not desire future vaginal

intercourse or vaginal preservation. The sexual-function consequence must be explicit. For the right patient, the numbers are strong: ACOG reports objective improvement around 98 percent, subjective improvement around 90 percent, low recurrence, and mortality about 0.15 percent. Many can be performed under local or regional anesthesia, which matters for patients whose cardiac, pulmonary, thromboembolic, or general medical risk makes longer reconstructive surgery less attractive. Colpocleisis can be the most patient-centered operation when the patient accepts the loss of vaginal intercourse and the goal is durable symptom relief with lower physiologic burden.

Hysteropexy means suspending the uterus rather than removing it. It is a viable alternative to hysterectomy for uterine prolapse, especially when fertility, body image, or the patient's view of hysterectomy matters. But the evidence base is less mature. ACOG cites mixed data: one study favored vaginal hysterectomy for recurrence, while a randomized trial found sacrospinous hysteropexy noninferior over 12 months, with apical recurrence plus bothersome bulge or repeat surgery at 0 percent versus 4 percent. Longer follow-up is needed. So teach hysteropexy as a real option, not an inferior afterthought, but counsel about less long-term certainty and continued uterine and cervical surveillance.

Mesh, recurrence, and selective use

Mesh and graft complications are not just "more surgical risk." Native-tissue surgery can bleed, infect, or cause voiding dysfunction. Mesh adds material-specific risks: contracture; exposure or erosion into the vagina; erosion into the urethra, bladder, or rectum; chronic pain; dyspareunia; and procedures to remove or revise the material. Because mesh is permanent, later surgery can be more complex.

That is why ACOG limits pelvic organ prolapse vaginal mesh repair to high-risk individuals in whom benefit may justify risk, such as recurrent prolapse, especially anterior or apical recurrence, or medical comorbidities that make more invasive and longer open or endoscopic procedures poor choices. The patient must give informed consent after reviewing benefits, risks, and alternative repairs. Consent must be concrete: possible improved anterior anatomic and subjective outcomes, but also exposure or erosion around 12 percent, repeat surgery for exposure around 8 percent up to 3 years, dyspareunia around 9 percent after vaginal mesh prolapse surgery, and possible erosion into vagina, urethra, bladder, or rectum.

Surgeon training is also part of patient safety. A surgeon using biologic graft or synthetic mesh for POP needs specific training in patient selection, anatomy, surgical technique, postoperative care, and management of complications. If those competencies are not present, referral to a surgeon with female pelvic medicine and reconstructive surgery expertise is the safer path.

Recurrent POP requires honest counseling before and after surgery. Recurrence is possible after any POP operation. ACOG reports recurrence rates between 6 and 30 percent. Women should be counseled about recurrence before the first operation, and recurrent POP should be evaluated like primary POP: symptoms, POP-Q anatomy, prior operative reports, prior exam, patient goals, and current risk. Many patients may choose observation or pessary rather than repeat surgery. If recurrent vaginal apex prolapse follows prior vaginal native-tissue apical suspension without durable support, options may include abdominal sacrocolpopexy, vaginal colpopexy with possible mesh or graft augmentation, or colpocleisis, depending on the patient and surgeon expertise.

Occult stress urinary incontinence

Significant anterior or apical prolapse can mask stress urinary incontinence. The mechanism is mechanical. The prolapse can obstruct or kink the urethra, so the patient may not leak while the prolapse is present. After prolapse repair restores anatomy, the obstruction disappears and stress incontinence appears. That is why ACOG recommends preoperative evaluation for occult stress urinary incontinence in all women with significant apical prolapse, anterior prolapse, or both, using cough stress testing or urodynamic testing with the prolapse reduced.

Every woman with significant apical prolapse, anterior prolapse, or both needs preoperative evaluation for occult stress urinary incontinence. The test must be done with the prolapse reduced, because the question is: what will the urethra do after the prolapse is no longer kinking or obstructing it? The options are cough stress testing with reduction or urodynamic testing with reduction. Reduction can be done with a pessary, swab, forceps, or speculum blade, but Novak gives the key technical warning: do not over-straighten the urethra into a false positive and do not obstruct it into a false negative.

Urodynamics is selective. ACOG does not say every POP patient needs it. It may help counseling when the patient has bothersome incontinence with Stage II or greater prolapse, or when there is voiding dysfunction. Stage II means the leading edge is within 1 centimeter above or below the hymen; beyond that, the anatomy is significant enough that it can mask, reveal, or interact with continence mechanisms. The purpose is counseling and planning: will treating the prolapse unmask stress incontinence, should a continence procedure be discussed, is there obstruction or emptying dysfunction? Order urodynamics when the answer could change the conversation or the operation, not as a reflex.

The continence procedure decision is a quantified tradeoff. In women without preoperative stress urinary incontinence undergoing abdominal sacrocolpopexy, the CARE trial found that adding Burch colposuspension reduced postoperative stress urinary incontinence from 57 percent to 34 percent. For vaginal POP surgery, OPUS

found that adding a prophylactic midurethral sling reduced postoperative stress urinary incontinence from 49 percent to 24 percent. But the additional procedure increases adverse effects, including bladder perforation, urinary tract infection, bleeding, and transient postoperative voiding dysfunction. So this is not automatic "add a sling." It is counseling: lower postoperative leakage risk in exchange for additional procedure-related morbidity.

Cystoscopy is not routine because every POP case is identical; it is routine when the operation puts the bladder or ureter meaningfully at risk. The classic example is uterosacral suspension, where a suture can kink or incorporate the ureter. The reason to look during the operation is simple: if the injury is found while the patient is still anesthetized, it can be corrected immediately instead of becoming a delayed urinary tract complication.

The cystoscopy has two jobs. First, survey the bladder completely for injury, suture, or mesh-related violation. Second, look for efflux from both ureteral orifices, because visible ureteral jets are the intraoperative sign that the ureters are patent rather than kinked or obstructed. It is done after the prolapse repair is completed but before anesthesia ends, precisely so a problem can still be fixed in the same operation.

Finally, remember the evidence hierarchy at the end of the bulletin. Level A means good and consistent scientific evidence. Level B means limited or inconsistent scientific evidence. Level C means primarily consensus and expert opinion. The strongest Level A anchors are these: uterosacral and sacrospinous native-tissue apical suspensions are similarly effective; synthetic mesh or biologic grafts do not improve transvaginal posterior repair outcomes; and anterior polypropylene mesh improves some outcomes but increases morbidity. Level B includes symptom-driven treatment, offering pessary, apical suspension at hysterectomy for uterine prolapse, sacrocolpopexy tradeoffs, obliterative procedures, unique mesh complications, and hysteropexy as an option with less evidence. Level C includes POP-Q documentation, pessary for future pregnancy, selective mesh use, specific mesh training, intraoperative cystoscopy for high-risk procedures, occult SUI evaluation, and counseling about concomitant continence procedures.

Final resident reflex

First, do not confuse anatomy with disease. POP-Q documents descent. Symptoms and functional burden determine whether the descent needs treatment.

Second, measure from the hymen. The hymen is zero. Negative is above it. Positive is beyond it. Stage I is more than 1 centimeter above the hymen. Stage II is within 1 centimeter of the hymen. Stage III is more than 1 centimeter beyond it. Stage IV is complete or near-complete eversion. Bulge symptoms become much more likely when the leading edge reaches about plus 0.5 centimeters, but that is a symptom threshold, not a staging boundary.

Third, think in compartments, especially the apex. If you treat uterine prolapse with hysterectomy and do not suspend the apex, you have removed the organ but not repaired the Level I support failure.

Fourth, offer conservative care seriously. A pessary is a real treatment, especially for women avoiding surgery or planning pregnancy.

Fifth, make every surgical decision a tradeoff conversation. Native tissue avoids mesh risk. Sacrocolpopexy may improve durability but adds abdominal and mesh-related complications. Obliterative surgery can be excellent for the right patient but sacrifices vaginal intercourse. Hysteropexy preserves the uterus but has less long-term evidence. Mesh may improve selected anterior or recurrent cases but carries material-specific complications and regulatory context.

Sixth, always look for hidden urinary risk. Reduce the prolapse and test for occult stress urinary incontinence. CARE and OPUS show that prophylactic continence procedures reduce postoperative leakage, but they also add morbidity.

If you keep those six reflexes, the topic becomes manageable. POP is common, nonmalignant, and often anatomically present without symptoms. POP-Q gives you the map. Symptoms give you the reason to treat. Apical support gives you the surgical logic. Mesh and continence procedures force quantified counseling. And the best management is not the most aggressive repair. It is the repair, pessary, observation, or counseling plan that matches the woman's anatomy, symptoms, risks, and goals.